A PLUS HOME HEALTH SYSTEMS
5670 W Flamingo Rd, Ste D, Las Vegas NV 89103
Ph: (702) 720-7929 | Fax: (702) 342-8398

Referral Form

Date: 4/8/26

Referral Source Information:

Referring Agency: NV Ortho Phone: (702) 902-4591
Physician Name: M. Rossidis, DPM

Patient Information:

Patient Name: James Helms Phone: (602) 763-7484
Patient Address: 1054 Plentywood Pl, Henderson NV 89002
Primary Diagnosis (ICD 10): I73.9 L97.522 L03.116 S90.822A

Primary Needs for Home Health (check all that apply):

- [ ] Skilled Nursing
  - [ ] Medication Management
  - [x] Wound Care
  - [ ] Diabetic Care
  - [ ] IV Antibiotics
  - [ ] Skilled Monitoring
  - Others: _____
- [ ] Physical Therapy
- [ ] Occupational Therapy
- [ ] Social Worker
- [ ] Home Health Aide

Other Pertinent Information: _____

MD Signature _____

Please FAX this form to (702) 342-8398

Further Instructions:
When we call, we will also need to obtain the following information before seeing the patient. If you have these available, feel free to fax along with this form.

- Patient demographic information (including insurance info)
- Recent H&P or recent progress notes
- Medication List

THANK YOU FOR YOUR REFERRAL!!!

NORTHWEST LAS VEGAS
7455 W. Washington Ave Suite 160
Las Vegas, Nevada 89128

Phone: (702) 878-0393
www.nevadaorthopedic.com

HENDERSON
1505 Wigwam Pkwy Suite 330
Henderson, Nevada 89074

Patient Information

Patient Name:	**JAMES HELMS**	**MRN#:**	2924420	**Date of Birth:**	08/30/1935
Social Security #:	161-30-7862	**Language:**	English	**Referred by:**	IFEANYI C. MADU, DPM
Gender:	Male	**Race:**	White	**PCP:**	IFEANYI C. MADU, APRN
Marital Status:	Undefined	**Ethnicity:**	Not Hispanic or Latino		
Home Address:	1054 Plentywood Pl Henderson, NV 89002-9250 Phone: (602) 763-7484 Mobile: (602) 763-7484 Email: mhelmsraper@hotmail.com	**Work Address:**	None	**Employer:** **Occupation:** **Employment Status:**	RETIRED None Retired
Guarantor Name:	JAMES HELMS	**Relation to Guarantor:**	Self	**Guarantor Information:**	1054 Plentywood Pl Henderson, NV 89002-9250 Phone: (602) 763-7484 Email: mhelmsraper@hotmail.com

Insurance

Information

Carrier Information	Plan Information	Subscriber Information
MEDICARE PO Box 6776 Fargo, ND 58108-6776 Phone: (855) 609-9960 Plan Type: Undefined	Group #: NONE Plan #: 4EH4MV8XC96 Enrollment date: 08/01/2000	JAMES HELMS DOB: 08/30/1935 1054 Plentywood Pl Henderson, NV 89002-9250 (602) 763-7484
TRICARE FOR LIFE PO Box 7890 Madison, WI 53707-7890 Phone: (866) 773-0404 Plan Type: Champus	Group #: NONE Plan #: 161307862 Enrollment date: 03/16/2020	JAMES HELMS DOB: 08/30/1935 1054 Plentywood Pl Henderson, NV 89002-9250 (602) 763-7484

Emergency Contacts

Michelle Helms, Child(602) 763-7484

7455 W. Washington Ave Suite 160
Las Vegas, Nevada 89128

Phone: (702) 878-0393
www.nevadaorthopedic.com

1505 Wigwam Pkwy Suite 330
Henderson, Nevada 89074

JAMES HELMS **MRN#:** 2924420 **DOB:** 08/30/1935, 90 years

Date: 04/07/2026

Orthopedic Evaluation

History of Present Illness

Patient words: 90-year-old male returns to the office with his daughter. Patient underwent revascularization with vascular surgery. His daughter continues to do wound care at home. He still has pain with significant swelling to the lower extremities.

The patient is a 90 year old male who presents for a follow-up visit.

Patient History

Allergies

No Known Drug Allergies
Allergies Reconciled

Medications

amLODIPine (2.5mg tablet, oral) Active.
Lipitor (80mg tablet, oral) Active.
Iron (ferrous sulfate) (325 mg(65 mg iro tablet, oral) Active.
Proscar (5mg tablet, oral) Active.
Coumadin (5mg tablet, oral) Active.
lisinopriL (40mg tablet, oral) Active.
Toprol XL (25mg Tablet, Extende, oral) Active.
clotrimazole (topical) Specific strength unknown - Active.
terbinafine HCL (250mg tablet, oral) Active.
traMADoL (50mg tablet, oral) Active.
furosemide (20mg tablet, oral) Active.
Medications Reconciled.

Past Surgical

Cardiac Valve Replacement
No pertinent past surgical history Phreesia 03/17/2026
Urinary and/or Bladder Surgery
Non-Contributory Past Surgical History

Social

Alcohol use: None 03/05/2024
Assistance Status: Home Assistance Available 03/05/2024
Current Rehab Status: No Current Drug Rehab 03/05/2024
Drug Dependency: No Drug Dependence 03/05/2024
Exercise: Rarely 03/05/2024
Hand Dominance: Right Handed 03/05/2024
Past Rehab Status: No Past Drug Rehab 03/05/2024
Pregnancy Status: Not Pregnant 03/05/2024
Stair Tolerance: <1 Flight 03/05/2024
Surgical Blood Transfusion: Approves surgical blood transfusion 03/05/2024
Tobacco use: Current Tobacco User, Smokes 1 pack of cigarettes per day 03/05/2024

Family

No pertinent family history Phreesia 03/17/2026

Problem List

Cellulitis of foot, left
Blister of foot, left

Vitals

04/07/2026 01:14 PM
Weight: 145 lb **Height:** 68 in
Body Mass Index: 22.05 kg/m²
Pain Level: 3/10

Blood pressure not pertinent to visit

Review of Systems

General Not Present- Chills and Fever.
Gastrointestinal Not Present- Diarrhea and Nausea.
Musculoskeletal Not Present- Atrophy.
Neurological Not Present- Loss of Consciousness.

Physical Exam

General: AAOx3, well nourished, no acute distress
Vascular exam: No palpable pulses to the left foot. On Doppler examination there was a monophasic signal to the both DP and PT arteries. Dependent rubor noted to the distal left foot. No varicosities noted, no pain to bilateral calves with palpation, capillary refill brisk x10, pedal hair was present bilaterally, no pitting edema noted bilaterally, normal skin temperature proximally to distally bilaterally
Neurologic examination: Epicritic sensation intact bilaterally, vibratory sensation was normal bilaterally
Musculoskeletal examination: Tenderness palpation to the left forefoot with edema
Dermatological examination: Toenails are significantly dystrophic, discolored with subungual debris x 10l
Left second toe on the medial aspect has a full-thickness ulceration down to adipose tissue. There is no significant drainage. Large serous blister noted to the dorsum of the left foot. No surrounding erythema.

Previously reviewed:
Radiographs: Left foot radiographs 3 views: No fractures or dislocations. No gas-forming organisms in the left foot.

General
Mental Status - Alert.
General Appearance - In no acute distress, Healthy and Normal appearance.
Orientation - Oriented X3.
Gait - Normal.

Neck
Neck - Normal.

Neurologic
Neurologic evaluation reveals - normal attention span and ability to concentrate, able to name objects and repeat phrases. Appropriate fund of knowledge , pupils equal and reactive to light and accommodation, normal sensation of trigeminal nerves, symmetrical functioning of facial nerves, normal hearing, normal gag reflex, symmetrical functioning of accessory nerves, symmetrical functioning of hypoglossal nerve, normal sensation, normal coordination and upper and lower extremity deep tendon reflexes intact bilaterally .

Musculoskeletal
General
Joints and Muscles - Normal joints and muscles.

Assessment and Plan

Peripheral artery disease (I73.9)
Toe ulcer, left, with fat layer exposed (L97.522)
Cellulitis of foot, left (L03.116)
Blister of foot, left (S90.822A)

Note: 90-year-old male with a history of acute on chronic peripheral artery disease with left second toe ulcer. Patient previously treated for cellulitis last visit with clindamycin which has improved. Patient's blister has been drained however continues to have significant serous drainage from the dorsum of the foot due to his edema. Patient was revascularized with stenting to the proximal vessels. I spoke to the vascular surgeon who performed the procedure stating the patient has significant disease within the SFA and the patient is not candidate for bypass surgery. He does have reconstitution of three-vessel runoff down to his foot however does have small vessel disease within the foot.
Although having better inflow to the lower extremity he still significantly at risk for limb ischemia to his foot. We will continue to palliate his wounds.

I have no plans performing amputation at this time. The patient will require skilled wound care and nursing at home. A referral for a at-home nurse to perform dressing changes every other day was placed with his insurance.

I recommended wound cleansing with Betadine gauze to the dorsum of the foot and iodoform to the medial aspect of the second toe every other day.

Comprehensive Ulcer Management Protocol

Medihoney dry sterile dressings were performed to the left second toe. I recommended the use of antibiotic ointment and Band-Aid to the left toe at this time.

- Pressure Reduction/Offloading
- Limit activities
- Elevate
- WBAT in sandals

- Infection
- Ulcer not acutely infected
- Cleanse ulcer daily with normal saline or antimicrobial cleanser during dressing changes
- Pt education on all local and systemic signs of infections including changes in size, swelling, drainage, odor, redness, pain, fevers, chills. Pt is instructed to call immediately should any develop

- Vascular status
- Now status post revascularization however the limb is still at risk due to his significant peripheral arterial disease.

- Metabolic Factors
- Discussed the importance of strict glycemic control as it's related to healing diabetic foot ulcers
-Increased protein uptake
- Nutritional Factors
- Reviewed the importance of healthy balanced diet and drinking water
- Advised to increase protein intake with Pro-Stat AWC or comprable supplement

Goal's:
- Reduce devitalized tissue
- Increased granulation
- Closure of ulceration
- Reduce drainage
- Reduce swelling
- Increase strength and mobility
- Improve glycemic control and increase protein

Orders
Medihoney Application

Additional Instructions
Follow up in 2 weeks

Electronically signed by: MICHAEL T. ROSSIDIS DPM
on 04/07/2026